

Community Stray Dog Care Guide

Comprehensive Veterinary & Community Knowledge Base
(Focused on Indian Street Dogs - Indies)

Version 6.0 | Expanded & Optimized for GraphRAG

A detailed, practical reference manual for community caregivers, feeders, and veterinary volunteers. Designed with consistent structure for high-quality knowledge graph extraction.

Section 1: Introduction & Safe Handling of Stray Dogs

This guide provides practical, evidence-informed knowledge for people working with free-roaming street dogs (commonly called Indies or Desi dogs) in India. It covers clinical information, safe interaction techniques, nutrition, first aid, population management, and community coordination strategies.

Understanding Indian Street Dogs (Indies)

Indian street dogs are highly adaptable, intelligent, and generally hardy animals. Most are medium-sized (12-25 kg), short-haired, and live in loose social colonies. They have well-developed communication systems using body language, vocalizations, and scent marking. They are resilient to India's harsh climates and variable food availability but face significant challenges from human conflict, traffic, disease, and malnutrition.

Key Characteristics

- High resilience to heat, rain, and inconsistent nutrition.
- Strong territorial instincts around reliable food sources.
- Generally wary of strangers but can form bonds with consistent, respectful humans.
- High reproductive rate — unspayed females can produce 2 litters per year with high puppy mortality.
- Excellent problem-solvers and scavengers.

Safe Approaches and Interaction Guidelines

Reading Body Language (Critical for Safety)

Relaxed / Friendly Signs: Loose, wagging tail with full body movement, soft eyes, relaxed or slightly forward ears, play bow, gentle approach, lying on side or back exposing belly.

Warning / Fearful / Aggressive Signs: Stiff body posture, raised hackles along the back, tail tucked tightly or held rigidly high, growling or snarling, direct hard stare, whale eye (whites of eyes visible), ears pinned back or aggressively forward, lip licking, yawning out of context.

Rule: If any warning signs are present — stop, create distance, and do not approach or touch the dog.

How to Safely Approach a Stray Dog

1. **Ask Permission:** Approach slowly from the side or at an angle (never straight on or from behind). Avoid direct eye contact initially. Lower your body by crouching or kneeling to appear smaller and less threatening. Speak in a calm, low, reassuring voice.
2. **Safe Petting Technique:** Let the dog initiate contact. Start by offering the back of your hand at chest or shoulder height (avoid reaching over the head). Use short, gentle strokes on the chest, shoulder, or side. Watch for signs of discomfort and stop immediately if they appear.
3. **Never Do:** Chase, corner, or grab a dog. Disturb a dog that is eating, sleeping, or nursing puppies. Put your face close to the dog's face. Approach or touch a dog that is visibly injured, in pain, or showing fear/aggression.

Safe Feeding Practices

- **Location:** Use consistent, open, easily cleanable areas away from heavy traffic and residential complaints.
- **Timing:** Feed at roughly the same times daily. This reduces anxiety and fighting.
- **Food:** Balanced homemade (60-70% cooked rice or roti + 20-25% protein like boiled chicken, dal, or eggs + vegetables). Commercial kibble is excellent when available. **Avoid:** Chicken bones, very spicy/oily food, chocolate, onions, garlic, grapes, excessive dairy, raw meat in high-risk areas.
- **Portions:** Adjust based on body condition. Overfeeding causes obesity and increases resource guarding.
- **Water:** Always provide clean, fresh water.
- **Multiple Stations:** In large colonies, use several feeding points to reduce conflict.

Basic Safety & Hygiene for Caregivers

- Wash hands thoroughly with soap before and after all interactions.

- Wear closed-toe shoes in colony areas.
- Maintain up-to-date tetanus and rabies vaccinations.
- If bitten or scratched: Immediately wash with soap and running water for 15+ minutes. Seek medical attention promptly for rabies post-exposure prophylaxis.
- Use gloves when handling sick, injured, or unknown dogs.

Section 2: Health Monitoring & Body Condition Scoring

Consistent observation turns everyday feeding into an early warning system for disease. Small changes noticed early can save lives.

Daily Observation Checklist

- **Appearance & Behavior:** Energy level, gait (limping?), interest in food/water, social behavior with other dogs and humans, coat quality (shiny vs dull/matted).
- **Visible Illness Signs:** Eye/nose discharge, coughing/sneezing, diarrhea (especially bloody), vomiting, visible wounds/swelling, excessive scratching, hair loss, lethargy.
- **Body Condition:** Can you see or easily feel the ribs? Is there a visible waist from above? Any sudden weight loss?

Body Condition Scoring (BCS) – 5-Point Scale for Street Dogs

Score 1 – Emaciated: Ribs, spine, and hips extremely prominent. No palpable fat. Severe muscle wasting. **Action:** Urgent feeding support + veterinary assessment.

Score 2 – Underweight: Ribs clearly visible. Pronounced waist. Some muscle loss. **Action:** Increase food quantity/quality and monitor closely over 1-2 weeks.

Score 3 – Ideal: Ribs easily felt with light pressure but not visible. Clear waist when viewed from above. Good muscle tone. **Goal** for healthy adult street dogs.

Score 4 – Overweight: Ribs difficult to feel under fat layer. Waist barely visible. **Action:** Reduce calorie-dense food and increase activity if possible.

Score 5 – Obese: Heavy fat deposits over ribs, no waist, rounded abdomen, difficulty moving. **Action:** Veterinary consultation for diet management and possible underlying issues.

Simple Record Keeping (Highly Recommended)

For dogs you see regularly, keep basic notes (notebook or phone): Dog description/photo + location, date, BCS, food intake, any symptoms observed, actions taken. These records become extremely valuable when combined with your GraphRAG system for timeline analysis and outbreak detection.

When to Seek Professional Veterinary Help

Immediate/Emergency: Severe lethargy or inability to stand, heavy bleeding, difficulty breathing, suspected poisoning, seizures, signs of severe pain.

Within 24-48 Hours: Persistent vomiting/diarrhea, refusal to eat >24-48 hours, suspected mange or heavy skin infection, visible tumors or large swellings, signs of distemper or parvovirus (especially puppies).

Section 3: Common Diseases & Conditions (Detailed)

This section uses a consistent structure optimized for knowledge graph extraction: Overview → Symptoms & Timeline → Transmission & Risks → Supportive Care → Prevention → When to Seek Urgent Help.

1. Canine Parvovirus (Parvo) – Highly Contagious & Often Fatal

Overview: Parvovirus is one of the most serious and common viral diseases affecting unvaccinated street dogs, especially puppies. It attacks rapidly dividing cells in the gastrointestinal tract and immune system, leading to severe dehydration, sepsis, and high mortality without intensive care.

Typical Clinical Timeline

- **Incubation (3-7 days after exposure):** Virus multiplies silently.
- **Days 1-3 (Early/Prodromal):** Sudden lethargy, loss of appetite, mild fever, depression. Owners often notice the dog is “just not right.”
- **Days 3-5 (Acute GI Phase):** Severe, frequent vomiting (often yellow/bilious), profuse bloody diarrhea with characteristic foul/putrid smell, rapid and severe dehydration, abdominal pain, high fever or hypothermia in advanced cases.
- **Days 5-7+ (Critical Phase):** Extreme weakness, sepsis, possible myocarditis (heart inflammation) especially in very young puppies. Without aggressive fluid therapy and supportive care, death often occurs from dehydration, shock, or secondary bacterial infection.

Transmission: Fecal-oral route. The virus is extremely hardy and can survive in the environment (soil, grass, shoes, bowls) for up to 12 months or longer. It is easily spread by contaminated hands, shoes, or objects.

Risk Factors: Puppies 6 weeks–6 months (highest risk), unvaccinated dogs, crowded or unsanitary conditions, stress, concurrent parasites.

Basic Supportive Care (While Awaiting/Transporting to Vet): Immediate isolation from other dogs. Keep warm and quiet. Offer small amounts of water or oral rehydration solution if not vomiting. Do not force food during active vomiting phase. Strict hygiene for caregivers.

Prevention: Core vaccination series starting at 6-8 weeks with boosters. Good hygiene at feeding stations. Prompt removal of feces.

Urgent Veterinary Help: ANY puppy or dog showing vomiting + bloody diarrhea or sudden severe lethargy. This is a true emergency — survival rates drop dramatically without intensive intravenous fluids, anti-emetics, antibiotics, and nutritional support.

2. Canine Distemper

Overview: A serious, highly contagious viral disease that affects the respiratory, gastrointestinal, and nervous systems. It can cause long-term neurological damage even in survivors.

Typical Clinical Timeline

- **Stage 1 – Respiratory/GI (Days 1-7 after exposure):** High fever (often biphasic), watery to purulent eye and nose discharge, coughing, sneezing, profound lethargy, anorexia.
- **Stage 2 – Gastrointestinal (Days 8-14):** Vomiting, diarrhea (sometimes bloody), thickening and hardening of paw pads and nose (“hard pad”).
- **Neurological Stage (can appear 1-3 weeks later or even months later):** Muscle twitching (especially facial), “chewing gum” seizures, circling, head tilt, partial or full paralysis, behavioral changes. Many dogs that survive the acute phase are left with permanent neurological deficits.

Transmission: Airborne droplets and bodily fluids. Highly contagious, especially in crowded conditions.

Risk Factors: Unvaccinated puppies and young dogs are most vulnerable.

Supportive Care: Isolation, warmth, nutritional support, antibiotics for secondary infections, seizure management if needed. No specific antiviral treatment exists.

Prevention: Core vaccination (DHPPi) starting at 6-8 weeks with proper boosters.

Urgent Help: Any neurological signs (twitching, seizures, circling) or persistent respiratory + GI illness in an unvaccinated dog.

3. Rabies (Zoonotic – 100% Fatal Once Symptomatic)

Overview: A fatal viral encephalitis. Once clinical signs appear, the disease is almost always fatal in both dogs and humans. It is a major public health concern.

Clinical Progression

- **Prodromal Phase (2-3 days):** Subtle behavioral changes, anxiety, fever, licking at bite wound site.
- **Furious (Excitative) Form:** Restlessness, aggression, biting at imaginary objects or people, excessive salivation ("foaming at the mouth"), hydrophobia (fear of water), disorientation.
- **Paralytic (Dumb) Form:** Progressive paralysis starting from the hind limbs or jaw (dropped jaw), difficulty swallowing, eventual coma and death.

Transmission: Primarily through bite wounds from an infected animal (saliva enters the wound).

Prevention (Most Important): Rabies vaccination for all dogs. Immediate thorough washing of any bite wound + medical attention for post-exposure prophylaxis (PEP) in humans.

Action: Any dog showing unexplained aggression, paralysis, or excessive salivation should be treated with extreme caution. Report suspected cases to local authorities/ABC teams.

4. Mange (Sarcoptic & Demodectic)

Sarcoptic Mange (Highly Contagious): Caused by *Sarcoptes scabiei* mites. Intense itching (pruritus), hair loss starting on ears, face, elbows, and hocks, red inflamed skin, crusting, secondary bacterial infections. Can spread rapidly to other dogs and cause temporary human skin irritation.

Demodectic Mange: Caused by *Demodex* mites (normal skin inhabitants that overpopulate in immunocompromised dogs). Patchy hair loss, usually not intensely itchy unless secondary infection occurs.

Management: Isolate affected dogs (especially sarcoptic). Medicated baths or spot-on treatments as advised by a veterinarian. Improve overall nutrition and immune support. Severe or widespread cases require professional veterinary care.

5. Tick-Borne Diseases (Ehrlichiosis, Babesiosis)

Overview: Common in Indian street dogs due to high tick prevalence. Transmitted by tick bites.

Acute Phase (7-21 days post-bite): Sudden high fever, lethargy, loss of appetite, swollen lymph nodes, pale gums, nosebleeds or small hemorrhages (petechiae), lameness.

Chronic Phase: Weight loss, severe anemia, hind limb weakness, eye inflammation (uveitis), bleeding disorders.

Management: Tick prevention and prompt removal. Veterinary-prescribed antibiotics (e.g., doxycycline) and supportive care. Regular tick checks during monsoon and summer are essential.

6. Leptospirosis (Zoonotic)

Overview: Bacterial disease affecting kidneys and liver. Transmitted via contact with urine of infected animals (rats, dogs) or contaminated water, especially during monsoons/flooding.

Signs: Sudden fever, vomiting, diarrhea, muscle pain, jaundice (yellowing of eyes/gums), increased thirst and urination, lethargy.

Action: Aggressive veterinary treatment with fluids and antibiotics is required. Avoid stagnant water. Zoonotic risk to humans — practice good hygiene.

7. Intestinal Parasites (Roundworms, Hookworms, Tapeworms)

Extremely common in street dogs. Signs include pot-bellied appearance (especially puppies), diarrhea, weight loss despite good appetite, visible worms in stool or vomit, anemia (pale gums), poor coat.

Prevention: Regular deworming every 2-3 months using a broad-spectrum product (e.g., containing praziquantel + pyrantel). Prompt feces removal from feeding areas reduces environmental contamination.

Section 4: Nutrition & Feeding Practices

Good nutrition is one of the highest-impact interventions community caregivers can provide.

Recommended Balanced Homemade Mix (Daily)

- 60-70% Carbohydrates: Cooked rice, roti, or millets (energy source).
- 20-25% Protein: Boiled chicken, eggs, lentils (dal), or plain curd (muscle repair, immunity, milk production in lactating females).
- 10-15% Vegetables: Pumpkin (fiber + Vitamin A), carrots, leafy greens (micronutrients).
- Small amount of healthy fat (ghee or chicken skin) for calories.

Safe Human Foods

Boiled chicken (no bones), plain rice, curd, bananas, pumpkin, carrots, boiled eggs, plain roti.

Strictly Avoid

- Chocolate (theobromine toxicity – can cause fatal heart arrhythmias).
- Onions & Garlic (oxidative damage to red blood cells – Heinz body anemia).
- Grapes & Raisins (acute kidney failure).
- Chicken bones (splinter and cause intestinal perforation).
- Very spicy, oily, or heavily processed human food.

Special Situations

- **Puppies:** Higher protein and more frequent meals (3-4 times/day).
- **Pregnant/Lactating Females:** Increased calories and protein.
- **Sick or Recovering Dogs:** Easily digestible food (rice + chicken broth), smaller frequent meals.
- **Senior Dogs:** Softer food, joint-supportive additions if possible.
- **Deworming:** Every 2-3 months recommended for street dogs.

Section 5: First Aid & Emergencies

Golden Rule: First aid buys time. It is never a substitute for professional veterinary care. Always seek vet help for serious conditions.

Basic First Aid Kit Recommendations

Clean gauze/cotton, bandages, antiseptic (Betadine or chlorhexidine), saline or clean water, tweezers (tick removal), gloves, scissors, adhesive tape, activated charcoal (poisoning – only if advised), thermometer, soft muzzle or cloth, blanket/towel for transport.

Common Emergencies – Quick Reference

Heatstroke (Very Common in India)

Signs: Excessive panting, bright red gums, drooling, weakness, collapse, vomiting.

Action: Move immediately to shade. Pour cool (not ice-cold) water over head, neck, and body. Offer small amounts of water. Transport to vet urgently.

Suspected Poisoning

Signs: Vomiting, diarrhea, seizures, sudden collapse, bleeding, unusual behavior.

Action: Do NOT induce vomiting unless specifically instructed by a veterinarian. Note possible toxin if known. Rush to vet immediately.

Trauma / Hit by Vehicle

Signs: Fractures, visible bleeding, inability to stand, shock.

Action: Minimize movement. Use a blanket or board as a stretcher. Control bleeding with pressure if possible. Immediate veterinary attention.

Difficulty Breathing

Signs: Labored breathing, blue/pale gums, open-mouth breathing at rest.

Action: Keep calm and minimize stress. Transport to vet immediately.

Seizures

Signs: Convulsions, loss of consciousness, paddling legs, jaw chomping.

Action: Clear the area to prevent injury. Do not put hands in mouth. Time the seizure. Seek veterinary care, especially if first seizure or prolonged.

Transport Tips

Use a blanket or towel to gently restrain fearful or aggressive dogs. Support the body fully when lifting. For aggressive dogs, seek experienced help or use a proper trap. Keep the dog warm if in shock.

Section 6: Community Management & ABC Program

The Animal Birth Control (Catch–Neuter–Vaccinate–Release) program is the most humane and effective long-term method for managing street dog populations in India.

Key Benefits of ABC

- Dramatically reduces unwanted litters and population growth over time.
- Decreases mating-related aggression and territorial fighting.
- Provides an opportunity for mass rabies vaccination.
- Improves overall health and longevity of the colony.

What Caregivers Can Do

- Identify females in heat or with puppies for priority sterilization.
- Support post-operative care and monitoring of dogs after ABC surgery.
- Maintain clean feeding areas to reduce complaints and disease.
- Build positive relationships with local residents and ABC teams.

Section 7: Behavioral Psychology & Social Dynamics

Resource Guarding: A stress response where a dog aggressively defends food, water, or a preferred spot. Common trigger: Perceived scarcity of resources. Management: Multiple feeding stations, consistent feeding times, avoid sudden movements near food.

Mating Season Aggression: Significant spike in testosterone-driven fighting among intact males when females come into heat. ABC greatly reduces this problem.

Section 8: Preventative Medical Protocols

Vaccination & Deworming Timeline (Recommended for Street Dogs)

- Age 4 weeks: Initial deworming (Pyrantel).
- Age 6-8 weeks: First DHPPi vaccine (Distemper, Hepatitis, Parvovirus, Parainfluenza).
- Age 10-12 weeks: Second DHPPi booster + First Anti-Rabies Vaccine.
- Age 6 months: Animal Birth Control (sterilization) surgery.
- Adult – Every 3 months: Broad-spectrum deworming.
- Adult – Annually: Anti-Rabies booster + DHPPi booster.

Section 9: Geospatial Mapping & Territory Tracking

Mapping dog populations and territories helps with systematic care and reduces feeding conflicts.

- **Home Range:** Most Indies operate within a 0.5–1.5 km radius. Regular presence creates a geospatial node.
- **Feeding Stations:** Geolocated hubs where multiple dogs converge daily.
- **Conflict Zones:** Overlapping boundaries between packs – high risk for fights and injuries.

Section 10: Feeder Network & Data Coordination

Individual feeders working in silos leads to inefficiency. Structured coordination improves outcomes.

- Schedule synchronization (morning vs evening shifts).
- Health status flags (missing dog for 48+ hours = localized alert).
- Dietary adjustment logging (medicated food for specific dogs).

Section 11: Zoonotic Disease Management

Primary Risks:

- **Rabies:** 100% fatal once symptomatic. Requires immediate Post-Exposure Prophylaxis (PEP) for humans.
- **Leptospirosis:** Transmitted via urine in stagnant water. Can cause severe human kidney/liver failure.
- **Ringworm (Dermatophytosis):** Fungal skin infection – easily treatable but highly contagious via direct contact.

Section 12: Neonatal & Pediatric Care for Street Pups

Puppy mortality on the streets often exceeds 60%. Specialized care is needed in the first 12 weeks.

- **Weeks 0-3 (Neonatal):** Completely dependent on mother for warmth and milk. Do not touch or move pups unless the mother is deceased or has abandoned them.
- **Weeks 3-6 (Weaning):** Introduce semi-solid food (mashed rice + broth). Highest vulnerability to Parvovirus as maternal antibodies decline.
- **Weeks 6-12 (Socialization):** Critical window for positive human interaction to prevent adult fear-aggression.

Section 13: Geriatric Street Dog Care

Dogs surviving past ~8 years are considered geriatric and need modifications.

- **Osteoarthritis:** Stiffness (worse in cold weather). Provide softer resting areas and consider joint supplements.
- **Dental Disease:** Severe tartar and tooth loss. Transition to soft, cooked meals.
- **Sensory Decline:** Vision/hearing loss increases risk of vehicular trauma.

Section 14: Seasonal & Environmental Stressors

- **Summer (Apr-Jun):** High heatstroke risk + explosion in tick populations.
- **Monsoon (Jul-Sep):** Leptospirosis spikes (flooded streets), severe fungal skin issues, gastrointestinal infections.
- **Winter (Dec-Feb):** Hypothermia risk in neonates and severe joint pain exacerbation in geriatric dogs.

Section 15: Incident Reporting & Relational Data Schemas

Structured logging enables powerful analysis in GraphRAG systems.

- **Dog Profile:** Unique ID, visual descriptors (coat, scars, ear notches), estimated age, sex, sterilization status, known health issues.
- **Encounter Log:** Timestamp, geolocation, observed BCS, symptoms, treatments administered.
- **Adverse Event:** Bite incidents, vehicle trauma, sudden disappearance – should trigger localized alerts across the feeder network.

Final Disclaimer

This guide is for informational and educational purposes only. It does not replace professional veterinary diagnosis, treatment, or advice. Dosages and medical interventions should only be administered under the guidance of a qualified veterinarian. Always prioritize the safety of both humans and dogs. In case of any bite or potential rabies exposure, wash the wound immediately and seek medical attention without delay.

Thank you for your compassion and dedication to community street dogs. Consistent, respectful care improves welfare for both dogs and the humans who share their space.